

Policy on Initial Assessment

«Hospital Name»

Document control

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Prepared by	Quality Coordinator Name: _____ Signature: _____		
Reviewed by	Quality Coordinator Name: _____ Signature: _____		
Approved by	Medical Superintendent Name: _____ Signature: _____		

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Statement of intent

Every patient undergoes a standardised initial assessment by qualified personnel within a defined time — producing a documented care plan for in-patients that the clinician in-charge authorises and that notes special needs after discharge.

1. Purpose

This policy says how «Hospital Name» performs standardised initial assessment for out-patients, day-care, in-patients and emergency patients; ensures assessment is done by qualified personnel within their scope of practice; completes assessment within a time frame based on patient need; includes nursing assessment at admission for day-care and in-patients; documents a care plan from the in-patient initial assessment; obtains countersignature by the clinician in-charge within 24 hours; and identifies special needs regarding care following discharge.

The chapter intent is that patients undergo initial assessment and periodic reassessments resulting in a care plan.

This policy does not cover re-assessment, registration, or clinical pathways for specific procedures — those are covered in other hospital policies.

2. Scope

This policy applies to treating doctors, nurses and other caregivers who perform initial assessment at «Hospital Name» in out-patient, day-care (including dialysis day-care), in-patient wards and emergency. It binds department heads who define who may assess, the Medical Superintendent, Medical Records, and the Quality Coordinator.

It covers the standardised initial assessment of out-patients, day-care, in-patients and emergency patients — including defined time frames for completion, nursing and medical assessment content, care plan preparation with appropriate countersignature, early identification of special needs, and the qualifications required to perform assessment.

Boundaries:

- Re-assessment intervals, care-plan modification after initial assessment, and early-warning escalation are covered in other hospital policies.
- That the patient is registered and admitted before or alongside assessment is covered in other hospital policies.
- Specific procedural pathways referenced by the guidebook are covered in other hospital policies; initial assessment for those pathways still uses this standard's formats.
- Lab and imaging own result reporting; this policy owns that available diagnostics inform the care plan.

3. Policy standards

«Hospital Name» performs initial assessment of out-patients, day-care, in-patients and emergency patients in a standardised manner. Assessment is performed by qualified personnel within their scope of practice, registration and applicable laws. Assessment is completed within a documented time frame based on patient needs — for in-patients within a maximum of 24 hours from arrival at the ward; for emergency from the time of arrival. Day-care and in-patient initial assessment includes nursing assessment at admission. The in-patient initial assessment results in a documented care plan. The care plan is countersigned by the clinician in-charge within 24 hours. The care plan identifies special needs regarding care following discharge.

«Hospital Name» does not leave an in-patient without a documented care plan, and does not treat the 24-hour maximum as a target when clinical need requires earlier assessment.

4. Non-negotiable rules

1. Do not use a non-standard assessment format in an area that has an approved standardised format for that area.
2. Do not omit vital parameters in emergency initial assessment; do not omit history, examination including vitals, drug allergies and provisional diagnosis for in-patients; do not omit medication reconciliation for in-patients.
3. Do not allow a person to perform an assessment outside their defined scope of practice and privilege.
4. Do not exceed the defined time frames — in-patient initial assessment maximum 24 hours from ward arrival; emergency from arrival to completion as defined.
5. Do not omit nursing assessment at admission for day-care and in-patients.
6. Do not continue planned in-patient treatment beyond emergency stabilisation without a documented care plan from the initial assessment.

7. Do not leave a care plan initiated by a junior doctor without countersignature by the clinician in-charge within 24 hours.
8. Staff who find assessment or care-plan rules not followed report it the same shift to the treating doctor or the Medical Superintendent.

5. What we do

5.1 Standardised initial assessment (OP, day-care, IP, emergency)

«Hospital Name» uses a standardised format for initial assessment in the OPD, day-care, in-patient and emergency areas. Formats may differ by department need but are the same within a particular area or unit. Emergency initial assessment includes recording vital parameters.

In-patient initial assessment covers history, examination including vital signs, documentation of any drug allergies, and provisional diagnosis. If a detailed assessment was done earlier the same day in OPD or emergency, it need not be rewritten in full; a comment links to the earlier assessment, and findings of all such assessments are reviewed and/or verified.

Initial assessment includes reconciliation of medications for in-patients. Abridged documentation may be used for day-care as appropriate, including patients coming for dialysis.

5.2 Performed by qualified personnel within scope of practice

«Hospital Name» determines who can do which assessment. Caregivers perform initial assessment within their scope of practice, registration and applicable laws and regulations. Psychological, spiritual, cultural, social and economic aspects of in-patient initial assessment may be done by various healthcare professionals as named on the privilege list.

Department heads maintain the assessment privilege list; the Medical Superintendent approves it.

5.3 Within a time frame based on patient needs

«Hospital Name» defines and documents the time frame for completing initial assessment for day-care, in-patients and emergency, and implements it.

For in-patients, the clock starts when the patient arrives at the ward and stops when initial assessment is completed. The maximum time is 24 hours. Unstable or high-dependency patients are assessed immediately, or within 10 minutes of arrival for potentially unstable cases, based on the Emergency Severity Index (ESI) triage framework — this is the organisation's adopted practice; NABH does not mandate a specific timeframe here. Routine ward patients are assessed within 24 hours of arrival, in line with the standard initial assessment window.

For emergency, the clock starts at arrival at emergency — primary survey and vitals immediately; triage begins immediately upon arrival, following the World Health Organization's Emergency Triage Assessment and Treatment (ETAT) approach — this is the organisation's adopted practice; NABH's own text specifies only that assessment be completed "from the time the patient arrives at the emergency until assessment is completed," with no specific number.

Day-care time frames are defined per service — before the planned procedure or dialysis session begins.

5.4 Nursing assessment at admission for day-care and in-patients

Initial assessment of day-care and in-patients includes nursing assessment at the time of admission, documented in the record. It identifies nursing needs and any special needs of the patient, completed within a defined time frame — within 24 hours of admission, aligned with the physician initial assessment window — this practice is consistent with comparable international hospital accreditation standards (e.g., JCI's requirement that nursing admission assessment be completed within 24 hours).

A checklist or template may be used. Abridged nursing documentation may be used for day-care as appropriate. Templates may be specific to the speciality or type of admission.

5.5 Documented care plan from in-patient initial assessment

For in-patients, the initial assessment results in a documented care plan. The care plan is written by the treating doctor or a doctor member of the treating team in the patient record and is followed.

The care plan is based on the initial assessment and results of diagnostic tests if available. It includes provisional diagnosis or differential diagnosis, relevant diagnostic investigations when required, initial treatment suggested, and specific instructions if any. It reflects the desired results of the treatment, care or service.

5.6 Care plan countersigned by clinician in-charge within 24 hours

Treatment may be initiated by a junior doctor, but the care plan is countersigned and authorised by the treating doctor — the clinician in-charge — within 24 hours. Countersignature is dated and timed in the record.

If the clinician in-charge will be unavailable beyond 24 hours, the covering consultant named for that period countersigns and the Medical Superintendent is informed.

5.7 Special needs regarding care following discharge

The care plan includes identification of special needs regarding care following discharge. Identification is critical for groups such as extremes of age, restricted mobility, continuing nursing and rehabilitation needs, and assistance with activities of daily living.

«Hospital Name» begins identifying special discharge needs early in the assessment process — not only on the day of discharge. Nursing and medical assessments both contribute. Detailed discharge process is covered in other hospital policies; this step owns early identification inside the care plan.

6. Governance and responsibility

Role	Responsibility
Medical Superintendent	Accountable for standardised formats,

	privilege lists and time frames. Approves assessment formats and privilege lists.
Department heads	Maintain area-specific formats and who may assess. Ensure medication reconciliation and care-plan quality in their units.
Treating doctors / clinician in-charge	Perform or supervise initial assessment; document care plan; countersign within 24 hours; identify special discharge needs.
Junior doctors	May initiate assessment and treatment; must obtain countersignature within 24 hours.
Nurses	Complete nursing assessment at admission for day-care and in-patients; contribute special-needs identification.
Medical Records	Ensure formats are available in records; flag missing care plans or countersignatures in record review.
Quality Coordinator	Audits formats, time frames, nursing assessment, care plans, countersignatures and discharge-needs identification.

7. Quality monitoring (RCA → CAPA)

What is monitored:

- Standardised formats in use in OPD, day-care, IP and emergency; vitals in emergency; IP content and medication reconciliation complete.
- Persons who performed initial assessment match the privilege list.
- In-patient assessments completed within 24 hours of ward arrival; emergency within defined time from arrival.
- Nursing assessment documented at admission for day-care and IP sample.
- Care plan present for IP sample; countersigned by clinician in-charge within 24 hours.
- Special discharge needs identified in care plans for applicable patients.

Root-cause analysis is required when missing care plans or late countersignatures recur; the organisation's defined recurrence threshold applies, as documented in the hospital's Root Cause Analysis procedure.

This policy is reviewed annually, and sooner when formats or privilege lists change.

8. Training and staff acknowledgement

Doctors, nurses and other caregivers who assess patients are informed of this policy at induction and once a year after that. Training covers area formats, privilege limits, time frames, medication reconciliation, nursing assessment, care-plan content, 24-hour countersignature, and early identification of special discharge needs.

Staff acknowledgement

I have read the Policy on Initial Assessment of «Hospital Name». I will follow the processes described.

Name: _____ Designation: _____

Department / floor: _____ Date: _____

Signature: _____

(One row per staff member. The Quality Coordinator holds signed acknowledgements with clinical induction records.)

9. Distribution

Official master copy: office of the Medical Superintendent, «Hospital Name», with the Quality Coordinator.

Copies issued to: every clinical department; emergency; day-care; nursing administration; medical records; OPD.

The current version is available to all staff at the clinical policy file and, if the hospital keeps an intranet, at staff intranet / policies.

When a new version is issued, take old copies out of use.

10. Abbreviations

Abbreviation	Meaning
AAC	Access, Assessment and Continuity of Care (NABH Hospitals chapter)
CAPA	corrective and preventive action
COP	Care of Patients (NABH chapter)
HCO	Hospital (Full Accreditation programme under NABH Hospitals 6th Edition)
IP	in-patient
NABH	National Accreditation Board for Hospitals and Healthcare Providers
OE	objective element
OP / OPD	out-patient / out-patient department

11. Traceability to NABH HCO Full Accreditation 6th Edition AAC.4

This table is an index. It is not how the policy is organised. An asterisk in the Level column means documentation of the process is required.

OE	Level	Requirement	Where addressed	Responsible
AAC.4.a	CORE*	The initial	Section 3; 5.1	Department

		assessment of the out-patients, daycare, in-patients and emergency patients is done in a standardised manner.		heads (formats); treating doctors (complete); nurses (contribute); Quality Coordinator (hold formats)
AAC.4.b	Commitment*	The initial assessment is performed by qualified personnel.	Section 3; 5.2	Medical Superintendent (approve privilege list); department heads (maintain); caregivers (assess within scope)
AAC.4.c	Commitment*	The initial assessment is performed within a time frame based on the needs of the patient.	Section 3; 5.3	Treating doctors (complete on time); department heads (define specialty defaults); Quality Coordinator (audit)
AAC.4.d	Commitment	Initial assessment of daycare and in-patients includes nursing assessment, which is done at the time of admission and documented.	Section 3; 5.4	Nurses (perform and document); nursing administration (templates and time frames)
AAC.4.e	CORE	The initial assessment for in-patients results in a documented care plan.	Section 3; 5.5	Treating doctor or doctor member of treating team (document); Medical Records (presence in record)
AAC.4.f	Achievement	The care plan is countersigned by the clinician	Section 3; 5.6	Clinician in-charge / treating doctor

		in-charge of the patient within 24 hours.		(countersign); junior doctors (seek countersignature)
AAC.4.g	Excellence	The care plan includes the identification of special needs regarding care following discharge.	Section 3; 5.7	Treating doctors and nurses (identify early); discharge-policy owners (execute discharge planning)

12. Required Records / Evidence Checklist

Records the hospital holds under this policy, listed by objective element.

AAC.4.a — Standardised initial assessment

- Approved standardised initial-assessment formats for OPD, day-care, IP and emergency.
- Emergency records showing vital parameters recorded at initial assessment.
- IP records showing history, examination including vitals, drug allergies, provisional diagnosis and medication reconciliation.
- Day-care records using abridged documentation where appropriate (including dialysis).

AAC.4.b — Performed by qualified personnel

- Assessment privilege list naming who may perform which assessment.
- Credentials and registration evidence for privileged clinicians who perform assessment.
- Sample records showing the person who assessed matches the privilege list.

AAC.4.c — Within a time frame based on patient needs

- Written time-frame document for day-care, IP and emergency initial assessment.
- IP sample with ward-arrival time and assessment-completion time within 24 hours.
- Emergency sample with arrival time and assessment-completion time within the defined frame.
- Day-care sample showing assessment before procedure/session as defined.

AAC.4.d — Nursing assessment at admission

- Nursing-assessment templates/checklists for IP and day-care.
- Documented nursing assessments at admission identifying nursing and special needs.
- Defined time frame for completing nursing assessment and audit against it.

AAC.4.e — Documented care plan from in-patient initial assessment

- Documented care plans in IP records including provisional/differential diagnosis.
- Care-plan entries for relevant investigations, initial treatment and specific instructions.
- Audit sample of IP admissions with care plan present after initial assessment.

AAC.4.f — Care plan countersigned by clinician in-charge within 24 hours

- Care plans with dated/timed countersignature by clinician in-charge within 24 hours.
- Covering-consultant countersignatures when treating doctor unavailable, with Medical Superintendent notified.
- Audit log of late or missing countersignatures with corrective action.

AAC.4.g — Special needs regarding care following discharge identified

- Care-plan fields or notes identifying special post-discharge needs.
- Evidence identification began early in the assessment process, not only on discharge day.
- Sample of applicable patients with special needs flagged and handed to discharge planning.

13. References

- National Accreditation Board for Hospitals and Healthcare Providers (NABH), Guidebook to Accreditation Standards for Hospitals, 6th Edition — Access, Assessment and Continuity of Care (AAC), standard AAC.4.
- American College of Emergency Physicians (ACEP), Emergency Department Triage policy statement (ACEP position paper; member-restricted access, no freely accessible URL available)
- World Health Organization. (2005). <https://www.who.int/publications/i/item/9241546875> Emergency Triage Assessment and Treatment (ETAT) course.

Disclaimer

This document is a template prepared for the guidance of «Hospital Name» and must be reviewed, adapted and formally approved by «Hospital Name» before use. Every entry marked _____ must be completed before the document is issued.

The requirements in this document are accreditation requirements of the NABH Accreditation Standards for Hospitals, 6th Edition, not duties under a named Act of Parliament. This policy does not import the Consumer Protection Act, 2019, the Clinical Establishments Act, 2010, or the Mental Healthcare Act, 2017 as a checklist. Statutory duties that arise under other documents of «Hospital Name» remain those documents. «Hospital Name» is responsible for verifying any statutory duty that applies to it; this document does not constitute legal advice.

«Hospital Name» remains responsible for verifying that it is current and consistent with the edition of the accreditation standard against which it is being assessed. The clinical and technical content reflects recognised national and international guidance current at the date of preparation.

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